# Value-Based ACO Contract Automation

## DigiPharm enable the R1 Cluster to automate the management of its value-based ACO contract with the CNHI and HHC.

In Saudi Arabia, the public healthcare sector is restructuring towards an Accountable Care Model (ACO) to ensure timely and appropriate care, improve outcomes, eliminate waste and reduce healthcare expenditure. Under new contracting obligations, the CNHI and HHC seek oversight over the performance of clusters and providers.

### Challenges

Underpinned by the transition to an ACO model. The CNHI and HHC seek assurance that Clusters are properly managing facilities, contractual obligations are being met and beneficiaries are receiving the right care for their needs. However, managing performance of multiple stakeholders across multiple clusters can be complex and resource intensive.

### Approach

We deployed a value-based contract management automation solution that aggregated data directly from providers within the R1 Cluster to automate reporting for the Cluster according to its CNHI & HHC contract requirements. The platform further provided a contract workflow and conducted the exploratory data analysis (EDA) to satisfy the Cluster Performance Measurement Framework.

### Results

We successfully digitised the Cluster ACO contract and aggregated relevant Ada'a, MOH, HHC, and CNHI related KPIs to enable automated monitoring and reporting. The system enabled the identification of the best and worst performing facilities within R1 to support value improvement initiatives. We were able to identify savings and highlight services that did not provide 'value'. The system eliminated the complexity and administrative burden of reporting and reconciliation of the ACO contract.

### Learnings

The R1 Cluster currently sits between the 'managed' and 'standardised' level on the Data Maturity Index. In our GAP analysis we identified that the current systems in R1 do not allow for cost correlation between facilities and PLICS data. Automated data quality checks also need to be introduced to ensure costing accuracy. Activity data quality needs improvement to enable patient matching and data verification. Categorisation and coding of HAC data also requires improvement to enable a standardised approach for data analysis.